

HEALTHCARE COMPLIANCE · REFERENCE

Medical Debt Collection Compliance

HIPAA, the FDCPA, and the fast-changing state rules that now decide who can compliantly recover patient balances — a 2026 reference

Medical debt recovery has become as much a compliance discipline as a collections one. With roughly \$207 billion in medical debt outstanding and patient responsibility climbing every year — the average single-coverage deductible reached **\$1,886 in 2025**, up **43% over the past decade** and 17% since 2020, with roughly **a third of covered workers now facing deductibles of \$2,000 or more** (KFF 2025 Employer Health Benefits Survey) — the pressure to collect is real. And it is getting harder: **71% of providers report patient-collection timelines stretching past 30 days**, while the rules governing *how* providers may collect have tightened sharply, with penalties for getting them wrong that now include having the underlying debt declared void. This reference lays out the framework a health system's recovery partner must operate inside in 2026: the federal baseline of HIPAA and the FDCPA, the shifting credit-reporting landscape, the state restrictions already in force, the No Surprises Act obligations that attach to out-of-network balances, and the compliance-first practices that recover the most without creating exposure.

Executive summary

- **HIPAA and the FDCPA / Regulation F set the federal baseline** — a Business Associate Agreement, the “minimum necessary” PHI standard, and a validation notice within five days of first contact are non-negotiable.
- The **CFPB's national medical-debt credit-reporting rule was vacated** by a federal court on July 11, 2025 — but that did not settle the issue; it moved it to the states.
- **States are restricting medical-debt credit reporting fast** — Maryland (Oct 1, 2025) and Oregon (Jan 1, 2026) now bar it outright, with non-compliant debt at risk of being declared void and uncollectible.
- The **No Surprises Act** adds Good-Faith-Estimate and reasonable-rate obligations to out-of-network balances — documentation a compliant agency must maintain.

1. The federal baseline: HIPAA and the FDCPA

Two federal frameworks govern every medical-debt account before any state rule applies. HIPAA controls how protected health information is handled at every step: a recovery partner must operate under a Business Associate Agreement with the provider, hold staff to ongoing HIPAA training, and apply the “minimum necessary” standard — accessing only the specific health information required to resolve the balance, and no more. The FDCPA and its Regulation F govern the collection conduct itself, because patient debt is consumer debt: a validation notice with the debt amount, creditor and consumer rights must go out within five days of initial

contact, the consumer’s communication preferences (including opting out of phone calls) must be respected, and the account must be worked without harassment or misrepresentation. These are the table stakes; an agency that cannot demonstrate both is a liability, not a partner.

2. The credit-reporting landscape moved to the states

For years, credit reporting was a central lever in medical collections. That is changing. On **January 7, 2025** the CFPB finalized a national rule with two aims: to strip some **\$49 billion in medical bills from the credit reports of about 15 million Americans**, and to bar lenders from using medical information in credit decisions altogether. On **July 11, 2025** the U.S. District Court for the Eastern District of Texas **vacated the rule in its entirety**, holding that the CFPB had exceeded its authority under the Fair Credit Reporting Act. The federal ban is not in effect, and the CFPB has since signaled only continued monitoring of aggressive collection practices — but that did not end the trend; it decentralized it. States are now writing their own, stricter restrictions, and they carry real teeth:

STATE / LAW	EFFECTIVE	WHAT IT DOES
Maryland — HB 1020 (Fair Medical Debt Reporting Act)	Oct 1, 2025	Bars disclosing any portion of a medical debt to a reporting agency; contracts after the effective date must include the prohibition or risk voidability; previously-reported items had to be removed by Nov 1, 2025.
Oregon — SB 605	Jan 1, 2026	Prohibits reporting the existence or amount of medical debt to consumer reporting agencies, and applies to providers <i>and their collection agents</i> ; violations can render the underlying debt void and uncollectible; requires financial-assistance screening before collections.
Additional states (e.g. Vermont, Rhode Island)	Rolling	Enacting comparable medical-debt reporting restrictions; the map changes frequently and must be tracked per account’s governing state.

Because the rules vary by state and change often, accurate account typing and state fields at placement are now essential — and in states like Oregon and Maryland a recovery partner must avoid even the threat of credit reporting, which is now unlawful there.

3. The No Surprises Act and out-of-network balances

For out-of-network and certain self-pay balances, the No Surprises Act adds a further layer. Patients must have received a Good Faith Estimate before service, and collectors must be able to show that the charges pursued reflect reasonable market rates — supported by regional benchmark comparisons and documentation that can withstand an Office of Inspector General audit or an Independent Dispute Resolution proceeding. In practice this means a compliant agency maintains a documented log of patient communications and rate comparisons on these accounts, rather than simply pursuing the billed figure.

4. Why compliance is now the deciding factor

The through-line across all of this is that in 2026, compliance is not a constraint on recovery — it is the precondition for it. A misstep no longer just risks a dispute; in several states it can void the debt entirely and expose the provider to enforcement. That raises the bar on who a health system can safely place with: the partner must combine HIPAA and FDCPA discipline with a current, state-by-state view of medical-debt rules, correct account typing at intake, and a compliance-first recovery model — omnichannel outreach with promise-to-pay

tracking, patient education, flexible interest-free payment plans, and contingency-only billing. Done right, that approach recovers more precisely because it protects the provider’s reputation and its receivable at the same time.

5. Best practices that recover more — within the rules

With credit-report pressure off the table in a growing list of states, the agencies that recover the most are the ones that lean into patient education and friction-free payment rather than leverage. In practice that means explaining the insurance Explanation of Benefits so patients understand what they actually owe and why; offering interest-free, flexible payment plans; and meeting patients where they are — **62% of consumers say they prefer to pay medical bills online**, so a digital payment portal materially lifts resolution. It also means working accounts omnichannel — phone, email, and text — with every contact and promise-to-pay tracked in monitoring software, so nothing is over-worked and everything is documented. The gap this closes is real: in-house teams often carry higher denial and write-off rates, while a specialized recovery partner can hold denial rates as low as **2–5%** through disciplined coding accuracy and denial management — the difference between a balance recovered and a balance forfeited.

How Southwest Recovery de-risks medical recovery

We are built for this framework. Every healthcare client is covered by a Business Associate Agreement; our staff train continuously on HIPAA and FDCPA / Regulation F; and we maintain a current, state-by-state view of medical-debt credit-reporting rules, furnishing to Equifax and Experian only where it remains lawful and never where a state now prohibits it. Accounts are typed and state-coded at intake so the right rules apply from the first contact, out-of-network balances are documented to No Surprises Act standards, and every account is worked on a compliance-first, contingency-only basis — no upfront fee, no recovery, no fee. The result is recovery that stands up to audit and protects the provider as much as it returns revenue.

For a health system, that combination is the whole point: recovery that returns revenue without creating regulatory exposure. In a landscape where a single reporting misstep can void a debt in one state while being routine in another, a partner who tracks the rules per account — and can prove it under audit — is not a nicety; it is the difference between recovering a balance and forfeiting it.

What to have ready when placing medical accounts

Compliant medical recovery starts with a compliant placement file. Complete documentation shortens disputes, supports lawful furnishing where a state still permits it, and stands up if a regulator ever asks. For each account, assemble:

YOU PROVIDE	WHY IT MATTERS
Executed Business Associate Agreement	The HIPAA foundation — in place once, covering every placement that follows.
Itemized statement & account type	The balance, how it accrued, and whether it is insured residual, self-pay, or out-of-network — account typing at intake determines which rules apply from the first contact.
Insurance EOBs	The adjudication record that defines true patient responsibility — and the document that resolves most patient confusion.

YOU PROVIDE	WHY IT MATTERS
Good Faith Estimate (where applicable)	The No Surprises Act paper trail for out-of-network and self-pay balances.
Patient state & contact data	Drives the state-by-state rule set — reporting eligibility, call limits, disclosures — applied per account, not per portfolio.
Your collection-attempt log	Notices sent and calls made — the internal history that frames compliant next steps.

From a complete file, the cadence is immediate: accounts are typed and state-coded at intake, outreach begins within 24–48 hours under the applicable rule set, disputes and verification requests are handled on the statutory clock, and your revenue-cycle team watches every account move in the client portal — placements, contacts, promises-to-pay, recoveries and remittances, on demand. On contingency, the entire program runs at zero fixed cost: no recovery, no fee — and every recovered dollar arrives with the documentation to prove it was collected the right way.

The closing discipline mirrors the opening one: review the program quarterly on four numbers — recovery rate by account age, placement freshness, dispute and complaint counts, and net-back after fees. Those measures tell a compliance committee and a CFO the same story from two directions: the revenue is being recovered, and the method would survive any audit that asked how. In a rulebook that now differs state by state and changes year by year, that provable answer is the entire value of a specialist partner.

About Southwest Recovery Services

Southwest Recovery Services is a HIPAA-compliant medical and healthcare receivables recovery firm, working patient and payer balances on pure contingency since 2004 — for health systems, hospitals, physician groups, and specialty and post-acute providers. We operate under a Business Associate Agreement with every healthcare client, hold staff to ongoing HIPAA and FDCPA / Regulation F training, and furnish qualifying accounts to Equifax and Experian only where lawful (honoring the evolving state medical-debt reporting restrictions). Over two decades we have placed more than \$975M in receivables, recovered over \$76M for clients, and worked 800,000+ accounts nationwide from 12 offices in 7 states — on pure contingency, with no upfront fee, no retainer and no minimum, and a clean compliance record since 2004.

Talk through compliant medical recovery

We'll walk your patient-A/R file and show you how we recover it inside HIPAA, the FDCPA, and the current state medical-debt rules — on contingency, so there's no cost unless we collect.

Call **(866) 551-4684** · sdietz@swrecovery.com · swrecovery.com

Provided for general informational purposes; not legal advice. Statutes and credit-reporting rules for medical debt vary by state and change frequently — confirm current requirements with counsel. Recovery figures are illustrative and vary by portfolio. Southwest Recovery Services is HIPAA-compliant and does not collect consumer debt in California, Oregon or Washington.